INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Weekly cetuximab + FOLFOX-4

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
14 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00692
Version 2

A 14-day cycle with weekly cetuximab. Chemotherapy starts on Day 1 and continues on Day 2 through two separate 22-hour fluorouracil infusions. There is no fixed total number of cycles.

Treatment stages

01 **A 14-day reference cycle**
Clinic → home → review

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. For sudden chest pain, collapse or serious breathing difficulty, call 112/999 or seek emergency care immediately. Contact oncology urgently for severe diarrhoea, inability to drink or sudden illness. Report eye pain, blurred vision, blistering or infected skin promptly. Call for line pain, swelling, leakage or unexpected pump progress.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How your medicines work

The aim is to control advanced bowel cancer and symptoms. The team reviews how well treatment is working and whether side effects remain acceptable; there is no single fixed number of cycles for everyone.

EGFR-TARGETED ANTIBODY

Cetuximab

Erbitux



Blocks growth messages at the cell surface

Cetuximab attaches to EGFR on the outside of cells and blocks some growth messages. In this bowel-cancer plan, validated extended RAS testing helps identify whether treatment is suitable. Healthy skin also uses EGFR, so skin and nail effects need early care.

EGFR receives a growth signal → Cetuximab blocks the receptor → Some growth signals are reduced

What to watch for

Report rash or sore nails early. Tell the nurse immediately about infusion symptoms. Eye pain, new breathing problems or severe diarrhoea need prompt advice. Blood tests check minerals during treatment and for two months afterwards.

PLATINUM CHEMOTHERAPY

Oxaliplatin

Eloxatin



Links deoxyribonucleic acid (the cell’s genetic material) (DNA) strands

Oxaliplatin makes links within DNA. These interfere with copying and repair, which can stop a cancer cell dividing. Healthy tissues can also be affected.

Platinum enters cell → DNA links form → Copying is disrupted

What to watch for

Cold can trigger tingling or throat discomfort. Tell the team about persistent numbness, difficulty with buttons or walking, or infusion symptoms. New breathing difficulty always needs assessment.

CHEMOTHERAPY MODULATOR

Folinic acid

Calcium leucovorin



Strengthens fluorouracil action

Folinic acid helps fluorouracil remain bound to an enzyme needed to make DNA. In this regimen it increases fluorouracil activity and can also increase its unwanted effects. It is not a vitamin supplement to start yourself.

Folinic acid → Stronger enzyme binding → Fluorouracil effect increases

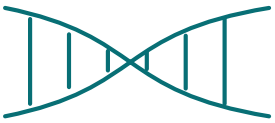
What to watch for

The combined treatment can increase mouth soreness and diarrhoea. Report these promptly. Rare allergic reactions can occur.

CHEMOTHERAPY

Fluorouracil

5-FU



Disrupts cell building blocks

Fluorouracil interferes with the production and use of DNA and RNA, the instructions cells need to grow. This FOLFOX4 schedule gives a bolus followed by a 22-hour infusion on Day 1 and repeats that fluorouracil sequence on Day 2.

Drug enters cell → DNA / RNA disrupted → Growth slows

What to watch for

Diarrhoea, mouth sores and skin soreness may become serious. Chest pain during the pump infusion needs emergency assessment. Unexpected severe early toxicity can occur even after dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) testing.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim

The setting changes what benefit means.

Why this option?

This combination is used for RAS wild-type bowel cancer that has spread. Your team explains the aim, alternatives and how benefit will be reviewed.

A plan that can change

Discuss how treatment may control the cancer or relieve symptoms, and whether the multidisciplinary team has a separate local-treatment goal. Your preferences and priorities matter. This regimen is for metastatic disease; it is not a routine postoperative recurrence-prevention course.

OPUS: exploratory extended RAS analysis

Studies can help explain why a treatment is offered. The schedule, previous medicines and tumour tests affect how well the evidence fits you.

Adding cetuximab to FOLFOX4 improved tumour shrinkage in a small selected trial group. That analysis did not establish a clear survival advantage. Tumour shrinkage is not the same as cure.

CHAPTER 02

Your schedule, tests and appointments

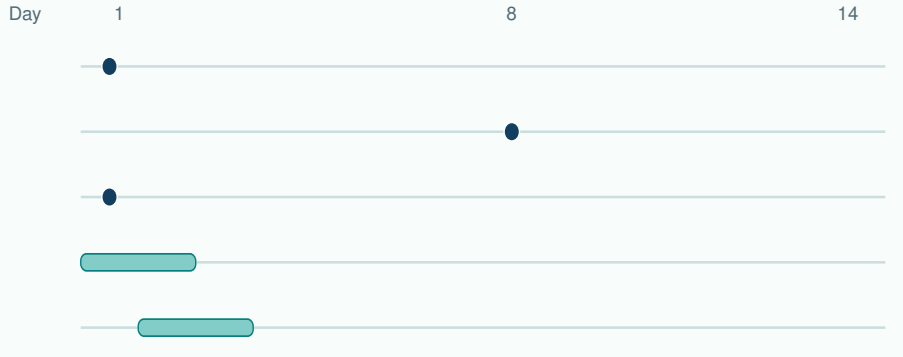
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment, recovery and review

Weekly cetuximab + FOLFOX-4

14-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

CLINIC → HOME → REVIEW

A 14-day reference cycle

D1 D2 D3 D8

Cetuximab on Day 1 and Day 8. FOLFOX4 starts on Day 1: oxaliplatin, folinic acid and fluorouracil bolus, followed by a 22-hour fluorouracil infusion. Day 2 brings another fluorouracil bolus and a second 22-hour infusion; finish around Day 3. Your team confirms the full Day 2 prescription. Repeat every 14 days while beneficial and tolerable.

A repeating reference cycle sits within a longer treatment plan. The oncology team chooses the number of cycles and confirms changes; no patient dates are required here.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

The columns show order, not equal periods. There is no fixed number of cycles for these plans. Agree who books each check, when results are reviewed and whom to contact if a check is missed.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00692 v2 · official regimen · EMA: Erbitux product information and patient leaflet

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Weekly cetuximab + FOLFOX-4: treatment and checks

Advanced / metastatic colorectal cancer

Cetuximab on Day 1 and Day 8. FOLFOX4 starts on Day 1: oxaliplatin, folinic acid and fluorouracil bolus, followed by a 22-hour fluorouracil infusion. Day 2 brings another fluorouracil bolus and a second 22-hour infusion; finish around Day 3. Your team confirms the full Day 2 prescription. Repeat every 14 days while beneficial and tolerable.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Repeated treatment	Longer-term review	Pause / completion
Blood counts, kidney and liver NCCP 00692	Baseline blood count, kidney and liver profile	Review results before treatment	Blood count, kidney and liver tests before each cycle	Repeat sooner if symptoms or results need it	Follow any unresolved abnormal result
Magnesium, calcium and potassium NCCP 00692	Check mineral levels before starting	Repeat periodically during treatment	Keep blood-test and replacement appointments	More checks may be needed if levels fall or diarrhoea develops	Monthly checks for two months after the last antibody dose
Skin, nails, eyes and breathing NCCP 00692	Discuss existing skin, eye or lung problems	Start the prescribed skin-care plan; report reactions early	Review skin, nails, diarrhoea and new symptoms	Eye pain, worsening breathing or severe rash needs prompt assessment	Continue care until symptoms and mineral levels settle
Fluorouracil safety and line NCCP 00692	Confirm the DPD test and safe line/pump arrangements	Check mouth, bowel and pump symptoms early	Review line, pump and previous-cycle toxicity	Report chest pain or unexpectedly severe early side effects immediately	Confirm disconnection and ongoing line care
Heart history and new symptoms NCCP 00692	Review heart history; electrocardiogram (ECG) if cardiac function is compromised	Report chest pain or new breathlessness immediately	Review new symptoms before further treatment	Further heart tests when clinically indicated	Follow any heart problem; no routine serial echo timetable

Care / check	Before starting	Early treatment	Repeated treatment	Longer-term review	Pause / completion
Nerve symptoms NCCP 00692	Record any numbness or tingling before treatment	Cold sensitivity can occur early	Formal neuropathy assessment every two cycles	Report persistent symptoms or difficulty with daily tasks before the next dose	Follow symptoms after oxaliplatin stops
Cancer response and treatment choices NCCP 00692	Record baseline disease and agree review plan	Bring forward review if symptoms worsen	Scans and other tests follow your consultant's treatment plan	Discuss benefit, side effects and whether the goal has changed	Agree continuation, a break, another treatment or supportive care

What each check is for

1 **A 14-day reference cycle**

Clinic → home → review

The aim is to control advanced bowel cancer and symptoms. The team reviews how well treatment is working and whether side effects remain acceptable; there is no single fixed number of cycles for everyone.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your port and treatment pump

From preparing the line to coming home after disconnection. Your team arranges the appointments and shows you what to do.

1 · Before treatment

Choose and prepare the line

The team explains whether a port or peripherally inserted central catheter (PICC) suits your treatment. You receive instructions for the insertion appointment.

2 · Insertion and recovery

A separate appointment may be needed

For a port, numbing medicine is used and a small device is placed under the skin. Ask about sedation, transport and wound care.

3 · Day 1

First 22-hour infusion

After the day-ward medicines and fluorouracil bolus, trained staff connect the first 22-hour infusion.

4 · Day 2

Second treatment and infusion

Return as arranged for further fluorouracil bolus and a second 22-hour infusion. The team confirms the setting and any other Day 2 medicines.

5 · Around Day 3

Professional disconnection

Staff check completion of the second infusion and care for the line. Do not use a single 46-hour pump timetable.

6 · Between cycles

The access device usually stays

A port needle can be removed while the port stays under the skin. A PICC remains in the arm with ongoing dressing and line care.

Port or PICC: what is the difference?

A port is a small chamber beneath the skin, usually on the chest, attached to a tube ending in a large vein. A special needle goes through the skin into the port when it is used. A PICC enters a vein in the upper arm; a short section remains outside under a dressing. Both can deliver a home infusion. Your team helps choose the device and teaches its care.

Before insertion and during recovery

Tell the team about allergies, blood thinners, infections and any other cancer medicines. Follow the instructions about eating, drinking and transport; do not stop medicines yourself. Port placement uses local anaesthetic, sometimes with sedation or general anaesthetic. The team explains the procedure and risks, including infection, bleeding, clots and insertion complications. Ask how to manage discomfort, protect the wound and resume activity. Report increasing pain, redness, swelling, discharge or fever.

Connection: what the nurse does

The nurse checks the line and treatment, secures the connection and dressing, and starts the pump. Before leaving, you should have the disconnection location and time, contact numbers including out of hours, and the written spill instructions. Ask to be shown what normal pump progress looks like. You do not need to calculate a rate.

Carrying it, sleeping and washing

Use the supplied carrier and the position recommended for your pump. Keep tubing free from pulling and kinks. Ask how to place the pump safely for sleep and how to protect the line and dressing when washing. Do not immerse it or swim. Avoid direct heat and cold. An elastomeric pump has a balloon that shrinks slowly and may have no alarm; an electronic pump has different instructions. Follow the instructions for the device you actually receive.

Disconnection is not port removal

Keep the appointment even if the pump looks empty, unless the team changes the plan. Staff check the pump and line and arrange safe disposal. After flushing and other line care as required, the port needle may be removed and a small dressing applied. The implanted port stays in place until a separate removal procedure is arranged. A PICC remains in place and needs scheduled dressing and flushing care.

Leakage, disconnection or unexpected pump progress

Call the team immediately if the pump leaks, is damaged, finishes unexpectedly early, appears not to run, or the line disconnects. Follow the emergency or spill steps you have been specifically taught, including use of a clamp if instructed. Do not reconnect, change settings or disconnect the pump yourself. Keep other people away from leaked medicine. If medicine touches skin, rinse it off and wash with soap and water, then seek advice.

Pain, swelling or feeling unwell

Contact oncology immediately for pain or swelling around the line, a wet/loose dressing, redness, fever, shivering or suddenly feeling unwell. A painful swollen arm or neck needs urgent advice. Severe chest pain, major breathing difficulty or collapse: call 112 / 999. Do not wait for pump disconnection or the next clinic visit.

The drawings show general steps, not instructions for inserting or accessing a line. Your hospital confirms its own arrangements and the care instructions for your device.

Visits, access and care between treatments

STEP 1

Before the first visit

Confirm the exact regimen, tests, medicines, emergency number and treatment goal. Discuss which central line is suitable and arrange pump training.

STEP 2

At the treatment unit

The first weekly cetuximab infusion takes about two hours; later weekly infusions usually take about one hour. Oxaliplatin and folinic acid are usually infused over about two hours, before fluorouracil. Blood review, preparation, premedicines and observation add time; infusion time is not the total appointment length. Allow at least one hour of monitoring after each cetuximab infusion; chemotherapy starts no earlier than one hour after cetuximab finishes. Your team confirms the final visit and observation plan.

STEP 3

After the infusion

Return on Day 2 for another fluorouracil bolus and renewal of the second 22-hour infusion. The team confirms any other Day 2 medicines. Final disconnection is planned around Day 3. This FOLFOX4 plan is not a single 46-hour infusion.

STEP 4

Between doses

Attend the separate Day 8 cetuximab-only visit. This is not another irinotecan or oxaliplatin day. Use the written symptom and support plan; do not wait for the next appointment if urgent symptoms appear.

STEP 5

The next decision

Confirm the next visit after any delay or dose change. Treatment continues only while the oncology team judges benefit and tolerability acceptable. Follow-up mineral checks continue after the final EGFR-antibody dose where applicable.

Transport and daily life

Plan transport if antihistamines, symptoms or other medicines affect driving. Ask for practical, work, nutrition or emotional support.

If treatment changes

Ask which component has been held and which visits remain. A treatment delay does not automatically cancel blood tests, line care or symptom follow-up.

Before the first treatment

Checks reduce avoidable risk.

Blood tests and medicines

Review blood counts, kidney/liver tests, symptoms, allergies and all medicines and supplements. Confirm DPD testing before fluorouracil. Kidney impairment can affect uracil-based DPD testing interpretation. Tell the team about red-meat allergy or tick bites before cetuximab because these can be associated with infusion-reaction risk.

Preparing access

Arrange Day 2 infusion renewal and final disconnection around Day 3. This is two separate 22-hour infusions; do not use a 46-hour pump timetable.

The infusion visit

An antihistamine and a steroid are given before every cetuximab infusion, including the Day 8 visit. Allow at least an hour of monitoring after each infusion and at least an hour before chemotherapy starts. The first cetuximab infusion takes longer than later ones; your total appointment also includes checks and preparation.

Visits and treatment sequence

Know which steps happen in the clinic and at home.

The exact delivery plan

Cetuximab on Day 1 and Day 8. Day 1: oxaliplatin and folinic acid, then fluorouracil bolus and a 22-hour infusion. Day 2: further fluorouracil bolus and a second 22-hour infusion; completion around Day 3. A repeating 14-day cycle. The number of cycles depends on benefit, side effects and your treatment plan. Your team confirms the complete Day 2 prescription before treatment.

Radiotherapy relationship

This NCCP regimen does not contain a routine radiotherapy course. If radiation is recommended separately, ask whether it is before, during or after these medicines.

When treatment changes or ends

Follow-up continues.

Symptoms and monitoring

Continue the cetuximab skin plan and mineral checks for two months after the last dose. Persistent numbness after oxaliplatin needs follow-up. Ask which medicines to continue, when to seek help and how response will be assessed.

Next decisions

The team reviews benefit, side effects and your priorities. Supportive and palliative care can help alongside active treatment. Ask for a clear next-step and contact plan.

Changes to one part of treatment

Cetuximab and the chemotherapy may be held or changed separately. Ask exactly which medicines and appointments remain, including the Day 8 visit, pump care and blood checks. A cycle stamp records what was given; it does not authorise the next treatment.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Cetuximab infusion	 **Day 1** intravenous (IV)
Cetuximab: second visit	 **Day 8** IV
Day-ward chemotherapy	 **Day 1** IV
First fluorouracil infusion	 **Days 1–2** About 22 hours
Day 2 treatment / second infusion	 **Days 2–3** Further bolus + about 22 hours
Skin, nails and minerals	 **Days 1–14** Can build over successive cycles
Cold-triggered sensations	 **Days 1–5** Often early; timing varies
Persistent nerve symptoms	 **Days 1–14** Can accumulate over cycles
Blood counts / infection risk	 **Days 1–14** Blood tests and symptoms guide review

Treatment or check	Timing in this view
Energy and bowel symptoms	1 14 **Days 1–14** Variable timing

Your planned visits: Cetuximab on Day 1 and Day 8. Day 1: oxaliplatin and folinic acid, then fluorouracil bolus and a 22-hour infusion. Day 2: further fluorouracil bolus and a second 22-hour infusion; completion around Day 3. A repeating 14-day cycle. The number of cycles depends on benefit, side effects and your treatment plan.

Care continues: Continue the skin-care plan and mineral checks arranged after cetuximab. Numbness or tingling can persist after oxaliplatin; tell your team if it affects daily tasks. Ask which medicines to continue and when your next review is planned.

No guaranteed safe days: An infection, reaction or other urgent problem can occur at any time. The graph does not predict a personal low point.

Why these bands stay broad: NCCP defines the treatment days. Symptom bands are broad educational guidance; reactions can occur on any day, and cumulative effects may continue between cycles. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00692 v2 · official regimen ↗

There are no guaranteed "safe days". Fever, shivers or suddenly feeling unwell require immediate contact. Serious symptoms override any timeline or diary.

Between-treatment notes

Day 1: Treatment and checks

The team reviews blood tests, symptoms and your prescription before treatment. Premedicines and observation are part of cetuximab visits. Know the contact number and the next planned visit.

Day 2–3: Day 2 treatment and completion

A second fluorouracil treatment is planned on Day 2; trained staff renew the infusion. The team confirms the setting and any other Day 2 medicines. Call early for symptoms; do not wait for the next appointment.

Day 4–14: Recovery and the next review

Weekly cetuximab includes a Day 8 visit between chemotherapy cycles. The team uses symptoms, tests and your priorities to decide what happens next. Bring questions and a symptom record if useful; this is optional.

Support around the medicines

1 Skin, nails and minerals

Start the prescribed skin-care plan early, use moisturiser and sun protection, and report painful or infected skin or nails. Keep mineral blood-test appointments during treatment and for two months after cetuximab.

2 Bowel symptoms and fluids

Follow your written rescue plan. Fever, blood, severe pain, reduced urine or inability to drink needs urgent advice.

3 Comfort and daily life

Avoid cold drinks and cold exposure as instructed after oxaliplatin; do not suck ice chips during fluorouracil in this regimen. Use prescribed anti-sickness medicines, pace activities and ask for nutritional, emotional or practical support.

4 The whole treatment plan

Ask how and when the team will assess benefit, which blood tests continue during a pause, and what happens after the final dose. Mineral checks continue monthly for two months after the last antibody treatment.

5 The cetuximab-only visit

Day 8 contains cetuximab only. Its premedicines and observation still matter, but the chemotherapy-day anti-sickness course is not automatically repeated.

Symptoms in context

Report what changed and how it affects you.

Living with treatment

Use the support available.

Pregnancy and fertility

Discuss fertility preservation and reproductive plans before treatment. Ask for the contraception and breastfeeding instructions for every medicine; the whole regimen may require longer precautions than one medicine alone.

Questions worth asking

Do I need to enter dates to understand the cycle?

No. The graph explains a general pattern. Your hospital arranges appointments; the guide and its safety information are available without entering anything.

What happens if a dose is delayed?

The team checks the cause and may delay, reduce, change or stop one medicine. Ask which component changes and when you should next attend; do not restart yourself.

How will I know whether it is helping?

Scans, symptoms and clinical review assess benefit. A rash, side effect or good day alone cannot show whether the cancer is responding.

Why is RAS testing mentioned?

RAS testing selects cetuximab treatment in colorectal cancer. It is not the selection test for the separate head-and-neck radiotherapy indication. Confirm the cancer-specific plan with your team.

Why is Day 2 different from other FOLFOX leaflets?

NCCP FOLFOX-4 has fluorouracil on Days 1 and 2, each with a 22-hour infusion. It is not modified FOLFOX-6. The team confirms your Day 2 prescription and visit; do not rely on another FOLFOX schedule.

Try explaining the plan back: Can you explain which visits are planned, whether a pump is involved, and which symptoms mean you should call today? No dates or symptom scores are needed to use this guide.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · chemotherapy and antibody

Active medicines: Cetuximab + Oxaliplatin + Folinic acid + Fluorouracil

Emetogenic risk: Moderate

Medicine	Dose / route	When
Chlorphenamine	10 mg intravenous (IV) bolus	At least one hour before cetuximab Repeat for subsequent cetuximab infusions; assess prior infusion reactions.
Dexamethasone	8 mg IV bolus	At least one hour before cetuximab on Day 1 This corticosteroid also covers the treatment-day corticosteroid part of antiemesis where appropriate. Pharmacy must reconcile route, dose and timing; do not automatically add a second 8 mg antiemetic dose.
Ondansetron	16 mg PO once	Before Day 1 chemotherapy NCIS V8 moderate-risk row; consider QT risk, low electrolytes, interactions and the individual prescription.
Dexamethasone	8 mg PO daily	Days 2 and 3 after the administered Day 1 chemotherapy NCIS V8 delayed prophylaxis. Reconcile with any other steroid prescription; re-anchor dates if treatment is delayed.
Metoclopramide	10 mg PO up to three times daily as required	Rescue nausea treatment only as prescribed NCIS V8 rescue option; check contraindications, renal function, interactions and prescribed duration.

Additional phase-specific support

- Antiemetic classification refers to the active treatment day. Reconcile duplicate steroid entries before prescribing.
- No automatic granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) schedule is added; use the patient's neutropenia history, risk and consultant plan.
- Cetuximab premedication and infusion observation remain necessary even on an antibody-only day.
- Start local EGFR skin care; monitor magnesium, calcium and potassium before and periodically during therapy, then monthly for two months after the final dose.
- Cold exposure can precipitate oxaliplatin neurotoxicity; do not recommend ice chips during fluorouracil in this combination. Review neuropathy every two cycles and sooner if symptoms interfere with function.
- Confirm line care, pump contact details and disconnection arrangements; report chest pain, severe early fluorouracil toxicity or pump/line problems urgently.

- NCCP 00692 v2 p1 permits omitting observation after two tolerated cetuximab doses, whereas current EMA Erbitux SmPC sections 4.2/4.4 require monitoring during and for at least one hour after every infusion. Reconcile this conflict with oncology pharmacy/local policy; do not automatically waive monitoring. Chemotherapy must start no earlier than one hour after cetuximab ends.
- FOLFOX4 uses two separate 22-hour fluorouracil infusions on Days 1 and 2, with a second treatment visit; never substitute a single 46-hour modified-FOLFOX6 pump schedule.
- Cetuximab and FOLFOX may be delayed independently. Reconcile actual treatment days, component-specific recovery and previous dose changes before updating support prescriptions.
- Intolerable cetuximab skin reactions require interruption even when graded below 3 under the EMA product information; review infection and supportive skin care promptly.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Day 2 · FOLFOX4 fluorouracil; confirm complete prescription

Active medicines: Fluorouracil

Emetogenic risk: Low treatment-day risk; delayed Day 1 moderate-risk coverage still applies

Medicine	Dose / route	When
Dexamethasone	8 mg PO once	Day 2; already part of the Day 1 moderate-risk Days 2–3 plan Reconcile this single dose across both plans; do not issue an additional duplicate dose.
Metoclopramide	10 mg PO up to three times daily as required	Rescue only as prescribed Review persistent nausea, hydration and individual contraindications.

Additional phase-specific support

- NCCP 00692 v2 p1 lists fluorouracil bolus and a separate 22-hour infusion on Day 2, but lists folinic acid on Day 1 only. Confirm the intended Day 2 folinic-acid prescription with the consultant and oncology pharmacy; do not infer or silently add a Day 2 dose from a generic FOLFOX4 schedule.
- No repeat cetuximab or oxaliplatin is scheduled on Day 2.
- NCIS multi-day guidance: cover the risk on each administered treatment day and delayed risk after treatment, using a reconciled prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Day 8 · weekly cetuximab only

Active medicines: Cetuximab

Emetogenic risk: Low

Medicine	Dose / route	When
Chlorphenamine	10 mg intravenous (IV) bolus	At least one hour before cetuximab Cetuximab premedication remains applicable.
Dexamethasone	8 mg IV bolus	At least one hour before cetuximab Reconcile low-risk antiemesis with this corticosteroid; do not duplicate steroids.
Metoclopramide	10 mg PO up to three times daily as required	Rescue only as prescribed No automatic delayed steroid course is carried over from Day 1 chemotherapy.

Additional phase-specific support

- Cetuximab only: no irinotecan, oxaliplatin, fluorouracil or atropine is scheduled on Day 8.
- Continue skin care, electrolyte monitoring, infusion precautions and the urgent contact plan.
- Low-risk antibody-only antiemesis is distinct from Day 1 moderate-risk chemotherapy prophylaxis.
- NCCP 00692 v2 p1 permits omitting observation after two tolerated cetuximab doses, whereas current EMA Erbitux SmPC sections 4.2/4.4 require monitoring during and for at least one hour after every infusion. Reconcile this conflict with oncology pharmacy/local policy; do not automatically waive monitoring. Chemotherapy must start no earlier than one hour after cetuximab ends.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Nerves

Oxaliplatin

Cold-triggered tingling or difficulty with buttons/walking: report.

2 Mouth / swallowing

Fluorouracil

Pain or ulcers affecting eating or drinking need advice.

3 Breathing / reaction

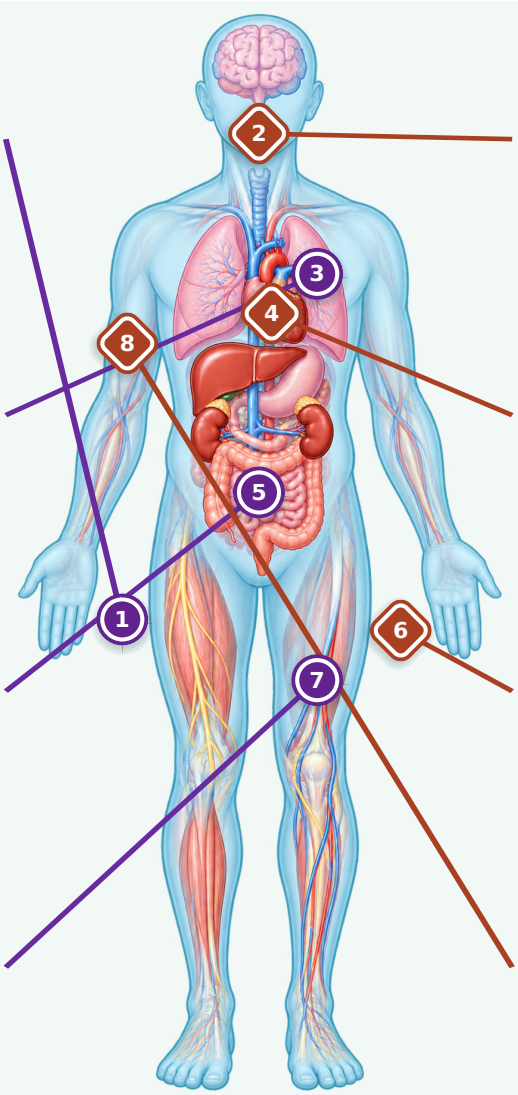
Cetuximab

New breathing difficulty requires prompt assessment.

4 Chest

Fluorouracil

New chest pain needs urgent assessment.



5 Bowel

Treatment

Diarrhoea, increased stoma output or dehydration: call early.

6 Skin / nails

Cetuximab

Report painful rash, cracks or infected skin early.

7 Blood / infection

Treatment

Fever, shivers or sudden illness: contact oncology immediately.

8 Line / pump

Treatment delivery

Pain, swelling, leakage or a loose connection needs immediate advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

For sudden chest pain, collapse or serious breathing difficulty, call 112/999 or seek emergency care immediately. Contact oncology urgently for severe diarrhoea, inability to drink or sudden illness. Report eye pain, blurred vision, blistering or infected skin promptly. Call for line pain, swelling, leakage or unexpected pump progress.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00692 v2 · official regimen**
   https://healthservice.hse.ie/documents/6532/692_v2_Cetuximab_FOLFOX4.pdf
2. **EMA: Erbitux product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/erbitux-epar-product-information_en.pdf
3. **eviQ: implanted port**
   https://www.eviq.org.au/p/507
4. **eviQ: peripherally inserted central catheter (PICC)**
   https://www.eviq.org.au/p/543
5. **eviQ: infusion pump care**
   https://www.eviq.org.au/p/502
6. **HSE: if you have side effects of cancer treatment**
   https://www.hse.ie/conditions/cancer/if-you-have-side-effects/
7. **OPUS extended RAS primary report**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC7508202/
8. **NCCP NCIS V8 multi-day systemic anticancer therapy (SACT) principles**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
9. **EMA: 5-fluorouracil dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) phenotyping and renal impairment (2024 safety communication)**
   https://www.ema.europa.eu/en/medicines/dhpc/5-fluorouracil
10. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
    https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq